

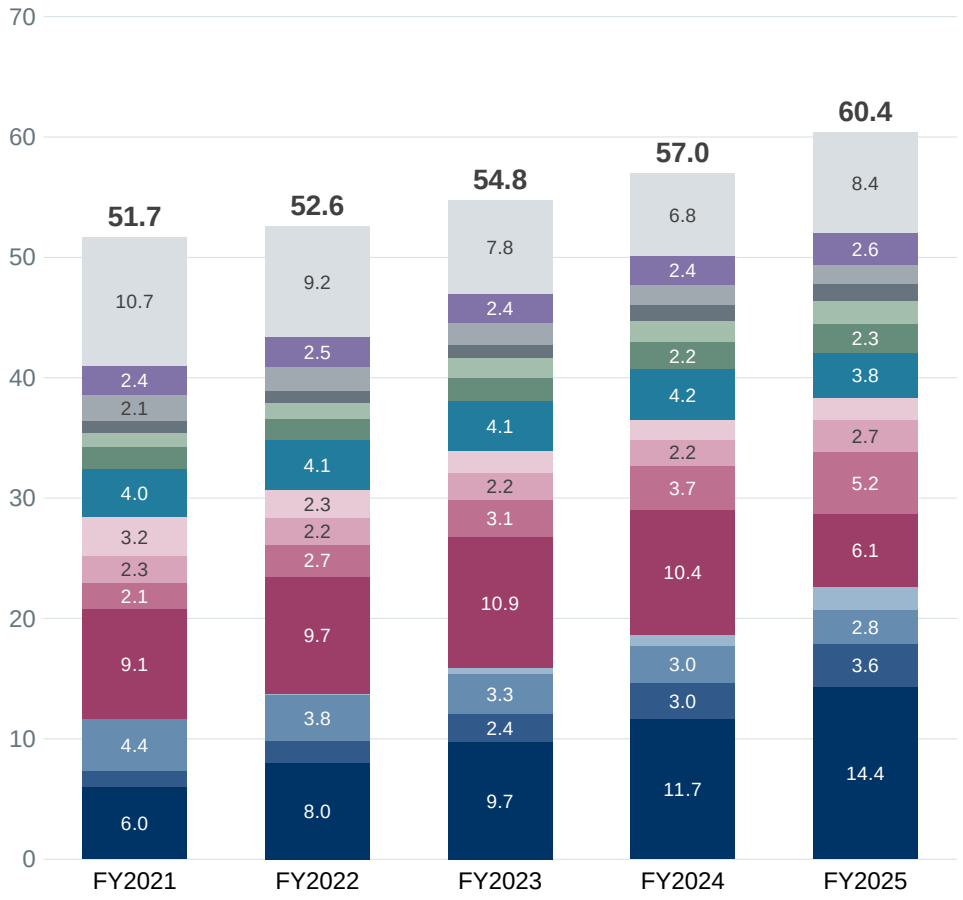
Agenda

1. Eli Lilly (US)
2. Johnson & Johnson (US)
3. AbbVie (US)
4. Roche (Switzerland)
5. AstraZeneca (UK)
6. Merck & Co. (US)
7. Novartis (Switzerland)
8. Pfizer (US)
9. Sanofi (France)
10. Bristol Myers Squibb (US)
11. Novo Nordisk (Denmark)
12. GSK (UK)
13. Amgen (US)
14. Takeda (Japan)
15. Gilead (US)
16. Boehringer Ingelheim (Germany)
17. Bayer (Germany)
18. CSL (Australia)
19. Viatris (US)
20. Teva (Israel)

Commercial portfolio: Oncology contributes 42% of FY2025 Innovative Medicine sales; growth offsets a sharp STELARA decline

J&J Innovative Medicine global sales | FY2021–2025

US$bn | total CAGR 4.0%



Core products / franchises

Product	FY25 $bn	CAGR	MoA
DARZALEX	14.4	+24%	Anti-CD38
ERLEADA	3.6	+29%	AR antagonist
IMBRUVICA	2.8	-10%	BTK inhibitor
CARVYKTI	1.9	n.m.	BCMA CAR-T
STELARA	6.1	-10%	Anti-IL-12/23 p40
TREMFYA	5.2	+25%	Anti-IL-23 p19
SIMPONI family	2.7	+4%	Anti-TNF
REMICADE	1.8	-14%	Anti-TNF
INVEGA LAI family	3.8	-1%	D2/5-HT2A antagonist
OPSUMIT/OPSYNVI	2.3	+6%	ETA/ETB ± PDE5i
UPTRAVI	1.9	+11%	IP receptor agonist
EDURANT	1.5	+11%	NNRTI
PREZISTA family	1.6	-7%	HIV PI-based regimens
XARELTO	2.6	+2%	Factor Xa inhibitor
Others*	8.4	-6%	Remaining products

* FY25 Others includes SPRAVATO ($1.7bn) / CAPLYTA ($0.7bn). Historical undisclosed product values not estimated.

Oncology
FY25 $25.4bn | 42.0%
2021–25 CAGR +14.9%

Immunology
FY25 $15.7bn | 26.0%
2021–25 CAGR -1.6%

Neuroscience
FY25 $7.8bn | 13.0%
2021–25 CAGR +2.9%

Pulmonary HTN
FY25 $4.4bn | 7.3%
2021–25 CAGR +6.5%

Infectious disease
FY25 $3.2bn | 5.4%
2021–25 CAGR -13.6%

CVM / Other
FY25 $3.8bn | 6.3%
2021–25 CAGR -2.1%

Portfolio / US LOE context

STELARA: biosimilar pressure
OPSUMIT: US generics in 2026
DARZALEX: licensed patents 2029
TREMFYA: composition patent 2031

[A22 p37; A23 p37; A25 pp37–38] USD reported sales. FY2021 restated; CAGR uses four intervals. Innovative Medicine only, excluding MedTech / Consumer Health.

Late-stage portfolio: Milestones centre on lifecycle expansion, with investigational co-antibody and FXIa programmes adding new mechanisms

10 Selected late-stage assets / programmes

8 Marketed assets with LCM milestones

3 Separate early clinical data programmes

TA	Asset / MoA	Modality	Origin / partner disclosed	2026 late-stage / regulatory context
Oncology	ERLEADA / apalutamide AR antagonist	Small molecule	Licensed: UC Regents / MSK	Localized / high-risk prostate cancer: ATLAS / PROTEUS
Oncology	TALVEY / talquetamab GPRC5D/CD3	TCE antibody	Genmab platform relationship	RRMM: CD38-naïve / exposed, MonumenTAL-3 / -6
Oncology	TECVAYLI / teclistamab BCMA/CD3	TCE antibody	Genmab platform relationship	MM 1–3 prior lines / CD38-exposed: MajesTEC-3 / -9
Oncology	RYBREVANT / amivantamab EGFR/MET	Bispecific antibody	Genmab platform relationship	Advanced head/neck cancer: ORIGAMI-4 US filing
Oncology	INLEXZO / gemcitabine system Gemcitabine DNA synthesis inhibition	Drug–device system	Origin not specified in supplied material	BCG-experienced HR NMIBC: SunRISe-5; naïve expansion discussed
Immunology	ICOTYDE / icotrokinra IL-23R antagonist	Oral peptide	Protagonist partnership	PsA ICONIC-PsA; UC / Crohn's Phase III per Q&A
Immunology	IMAAVY / nipocalimab FcRn blockade	Antibody	Momenta acquisition (2020)	wAIHA ENERGY (US approved in Q3); autoimmune expansion
Immunology	JNJ-4804 co-antibody IL-23 + TNF	Co-antibody combination	J&J guselkumab + golimumab programme	Refractory IBD: Phase III per Q3 Q&A
Neuroscience	CAPLYTA / lumateperone 5-HT2A/D2 pathway modulation	Small molecule	Intra-Cellular acquisition (2025)	Bipolar mania: ITI-007-451 / -452 US submission plan
CVM	Milvexian Factor XIa inhibitor	Small molecule	Partnered programme; partner not named here	AF / secondary stroke: LIBREXIA, event-driven

Early clinical data agenda

Bleximenib

Menin inhibitor
Relapsed/refractory AML; cAMeLot-1
Phase I/II data agenda

Pasritamig

KLK2/CD3 bispecific
Advanced prostate cancer; PCR1001
Phase I/II data agenda

JNJ-6143 / HLD-0915

RIPTAC induced-proximity platform
Prostate cancer; first-in-human update
Phase I / early clinical data agenda

NME under-review totals are not disclosed in these files. A planned submission is not treated as an accepted application.

[Q3P p27; Q3T pp13–15; Q2T p15] Selected programmes from supplied material, not a complete J&J pipeline or NME census. Milestones and asset stage are separate.

R&D priorities: Innovative Medicine prioritises Oncology, Immunology and Neuroscience, supported by new targeted and delivery platforms

Oncology

Earlier-line, multimodal regimens

- MM: BCMA / GPRC5D TCEs, anti-CD38 and BCMA CAR-T across treatment lines.
- Solid tumours: EGFR/MET bispecifics; prostate-cancer KLK2/CD3 and RIPTAC programmes.
- Bladder preservation: sustained intravesical delivery with broader risk-group development.

Immunology

Oral, dual-pathway and immune reset

- ICOTYDE (IL-23R oral peptide): PsA plus UC / Crohn's Phase III programmes.
- JNJ-4804 (IL-23 + TNF co-antibody): Phase III IBD for unmet need beyond monotherapy.
- FcRn autoimmune expansion and Sail in-vivo CAR-T collaboration.

Neuroscience

Broaden neuropsychiatric treatment

- CAPLYTA (5-HT2A/D2 modulation): adjunctive MDD and bipolar-mania development.
- SPRAVATO (NMDA antagonism): treatment-resistant depression franchise.
- Intra-Cellular acquisition adds CNS pipeline opportunities; not all stages disclosed.

Cross-TA platforms

Selected investment examples

- Ambrx: ADC platform; Halda: oral RIPTAC induced proximity.
- Firefly: degrader antibody conjugates; Sail: in-vivo CAR-T.
- INLEXZO / sustained erdafitinib: local delivery; not a fourth official TA.

Programmes, mechanisms and intended disease settings are shown; company opportunity estimates are not treated as clinical success or licensing demand.

[Q2P pp8,21; Q3P pp9,24,27; Q2T pp3–4,11–15; Q3T pp3–4,13–15; A25 p104] TA priorities are company-stated; platform examples are grouped analytically.

BD partnering: Oncology and Immunology account for six of seven named drug/platform transactions identified in the supplied reports

Therapeutic focus in the documented cohort

Oncology

3

Ambrx; Halda; Firefly

Immunology

3

Proteologix; NM26; Sail

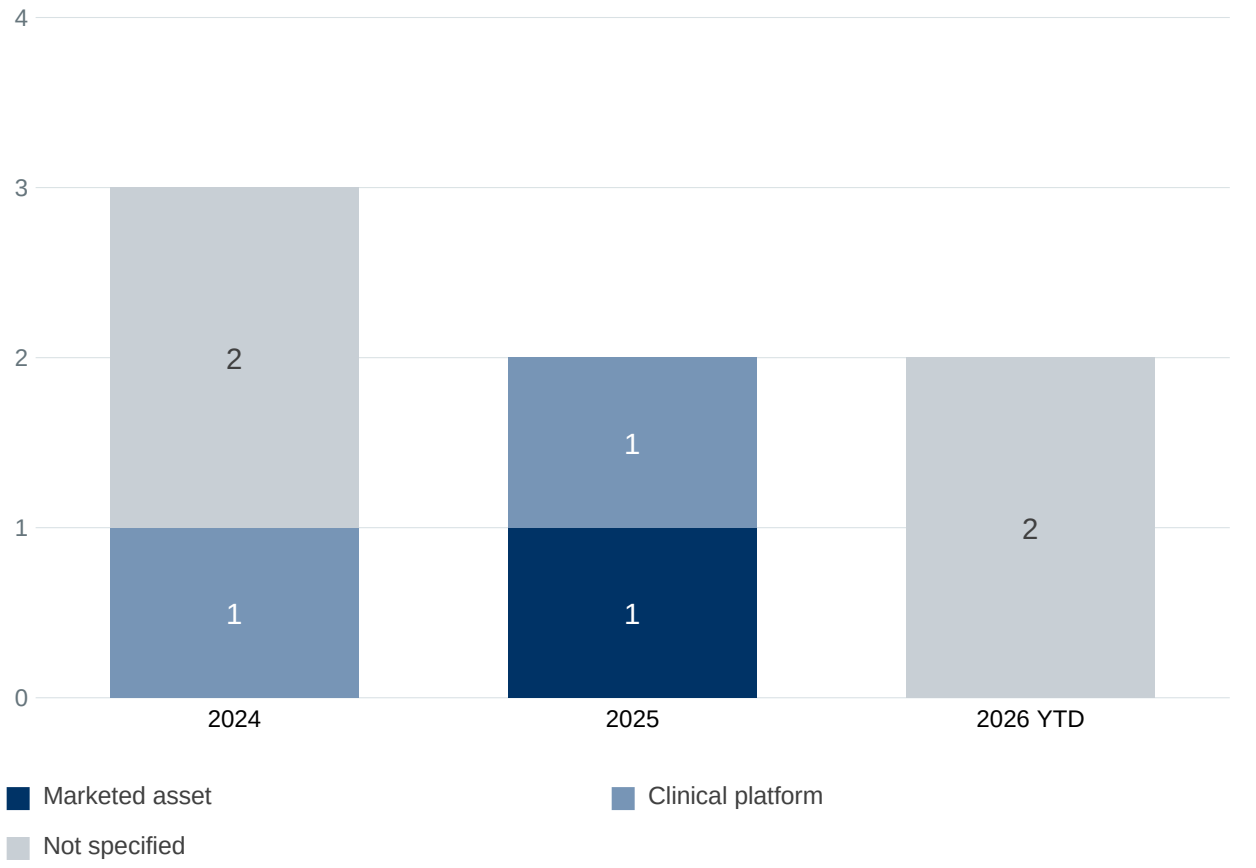
Neuroscience

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Intra-Cellular Therapies

Scope: named Innovative Medicine transactions disclosed in the supplied filings / calls. Earlier years without identified records are not plotted as zero.

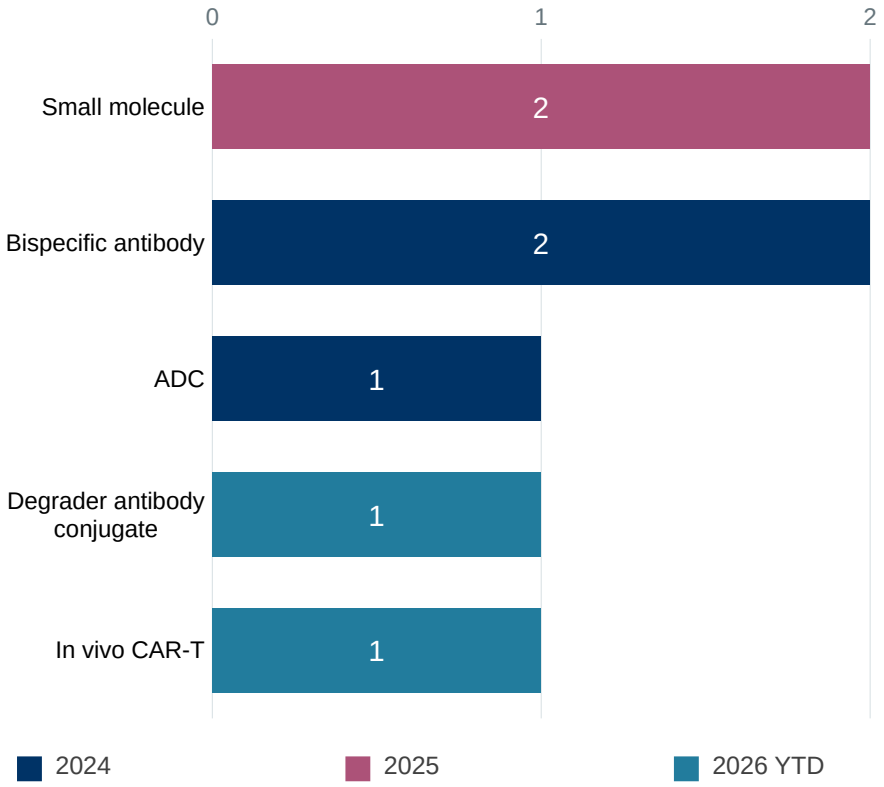
Broad development status at transaction date



[A25 pp104–106; Q2F pp33–34; Q3T pp4,7] Documented 2024–2026 cohort (n=7), not all J&J BD deals. MedTech, equity-only and R&D financing excluded.

BD partnering: The documented cohort spans small molecules, bispecific antibodies, ADCs, degrader conjugates and in-vivo CAR-T

Primary modality | number of documented transactions



One primary modality per record; a platform deal does not establish a specific target or clinical phase.

Targets / platforms supported by the reviewed sources

TA / year	Partner / asset	Target or mechanism disclosed
Oncology 2024	Ambrx Next-generation ADC platform	ADC targets not named in acquisition note
Immunology 2024	Proteologix Bispecific immune-disease portfolio	Not specified in reviewed filing
Immunology 2024	Yellow Jersey / Numab NM26	Two AD pathways; not named in filing
Neuroscience 2025	Intra-Cellular Therapies CAPLYTA + CNS pipeline	CAPLYTA: 5-HT2A/D2 pathways*
Oncology 2025	Halda Therapeutics RIPTAC; HLD-0915 / HLD-0117	RIPTAC targets not specified in acquisition note
Oncology 2026	Firefly Bio Firelink DAC platform	KRAS-driven cancers; precise target not disclosed
Immunology 2026	Sail Biomedicines In vivo CAR-T for immune-mediated diseases	Not disclosed in provided source

* Standard MoA annotation; not a quoted acquisition target. KRAS-driven tumour focus is not assumed to identify a specific antibody-binding antigen.

[A25 pp104–106; Q2F p33; Q3T pp4,7] Same seven-record cohort as prior page. One primary TA / modality per record; undisclosed targets are retained as undisclosed.

BD partnering: Two 2026 drug/platform transactions are identifiable in the supplied sources; JNJ-4804 co-funding is shown separately

Date / basis	Deal type	Counterparty	Asset / platform	Target / MoA	Modality	Stage at deal	Rights	Upfront / value
2026-06-08 Announcement	Company acquisition	Firefly Bio	Firelink DAC platform	KRAS-driven cancers; precise target not disclosed	Degrader antibody conjugate	Not specified	Company acquisition	$1.0bn cash
2026-07 Announcement month per Q3 call	Collaboration	Sail Biomedicines	In vivo CAR-T for immune-mediated diseases	Not disclosed in provided source	In vivo CAR-T	Not specified	Not disclosed	Not disclosed

Related 2026 financing | not counted as an inbound drug / platform transaction

Royalty Pharma × JNJ-4804

J&J can receive up to $0.5bn in 2026–2027 for clinical development. On qualifying approvals, Royalty Pharma may receive up to ~$0.5bn in fixed milestones, plus sales milestones / low-single-digit royalties. This is development financing, not J&J purchasing an external asset.

[Q2F pp33–34; Q3T pp4,7] Identified 2026 items through 13 Oct; not a full BD census. Financial terms shown only where disclosed. Co-funding is excluded from inbound deal counts.

Earnings calls summary: Forward R&D agendas highlighted by management and in Q&A

Selected asset, combination, indication, modality and delivery priorities | timing as stated at each call

Q3 2026 | 13 October 2026

Bring MM combinations into earlier lines

TECVAYLI (BCMA/CD3 TCE) + TALVEY (GPRC5D/CD3 TCE): management highlights second-line pivotal evidence as part of a treatment-to-cure strategy.

Segment oral monotherapy vs dual-pathway IBD

ICOTYDE (oral IL-23R peptide) for earlier advanced therapy; JNJ-4804 (IL-23 + TNF co-antibody) in Phase III for patients inadequately served by monotherapy.

Expand the bladder-preservation platform

INLEXZO (intravesical gemcitabine): expand beyond CIS into papillary / BCG-experienced and naïve disease through SunRISe-5 / -3.

Advance new prostate-cancer mechanisms

Q4 data expected for pasritamig (KLK2/CD3 TCE) and JNJ-6143 (RIPTAC oral platform), including the first Halda-derived clinical programme.

Add in-vivo CAR-T to immune-disease R&D

Sail collaboration advances in-vivo CAR-T for immune-mediated diseases; no antigen, clinical stage or specific study-start date disclosed in the call.

Follow event-driven FXIa outcome studies

Milvexian (FXIa inhibitor): secondary-stroke data potentially in 2026; AF data expected in 2027. Timing depends on events, not fixed enrolment dates.

Q2 2026 | 15 July 2026

Test dual TCEs in earlier-line myeloma

Await TECVAYLI (BCMA/CD3 TCE) + TALVEY (GPRC5D/CD3 TCE) data in RRMM after 1–4 prior lines, alongside earlier-line TECVAYLI + DARZALEX (anti-CD38).

Extend oral IL-23 into PsA and IBD

ICOTYDE (oral IL-23R peptide): PsA pivotal data expected in 2026; UC / Crohn's Phase III programmes target potential frontline advanced treatment.

Use local delivery across NMIBC risk groups

Expand INLEXZO (gemcitabine system) from CIS to papillary / BCG-naïve disease; develop sustained erdafitinib (FGFR inhibitor) delivery for intermediate-risk NMIBC.

Add induced-proximity and conjugate platforms

Halda-derived JNJ-6143 (RIPTAC) clinical data expected; planned Firefly acquisition adds a degrader-antibody conjugate platform for difficult cancers.

Broaden neuropsychiatric indications

CAPLYTA (5-HT2A/D2 modulation): bipolar-mania data expected in H2 2026, complementing the existing schizophrenia, bipolar-depression and adjunctive-MDD franchise.

Keep milvexian timing tied to clinical events

Milvexian (FXIa inhibitor): fully recruited outcome studies; AF timing remains event-driven, with no fixed readout date committed in Q2 Q&A.

[Q3T: 13 Oct 2026; Q2T: 15 Jul 2026] Uploaded LSEG edited transcripts. Speaker, page and exact excerpts retained in notes / companion evidence; no per-topic labels on page.